ONE HEALTH ANALYTICS · ZOONOTIC DISEASE SURVEILLANCE

The One Health Signal

Quarterly review of zoonotic disease surveillance

52
ISSUES PUBLISHED

45
CONSECUTIVE DAILY ISSUES

109
REGISTERED SOURCES

- The fastest-growing Ebola outbreak on record
- A second filovirus surfaces in Uganda
- H5N1 reaches the mammary gland
- Two mpox clades in one province
- Hantavirus goes to sea



Volume 1, Number 1 · Summer 2026 · Inaugural compilation

Covering **16 June – 30 July 2026**

Published by **One Health Analytics LLC** · Wilmington, Delaware

PUBLICATIONDATA

TITLE	The One Health Signal
VOLUME / NUMBER	Volume 1, Number 1 — Summer 2026 — inaugural compilation
PERIOD COVERED	16 June 2026 – 30 July 2026
DATE OF PUBLICATION	31 July 2026
PUBLISHER	One Health Analytics LLC · Wilmington, Delaware
EDITOR	Jae Seok Bae, DVM
FREQUENCY	Daily, weekly and monthly report series; compiled volumes issued quarterly
NEXT ISSUE	Volume 1, Number 2 — Autumn 2026 — 31 October 2026
WEBSITE	onehealth-analytics.org
ARCHIVE	archive.onehealth-analytics.org
SURVEILLANCE PLATFORM	gww-web.onrender.com
SUBSCRIBE	subscribe.onehealth-analytics.org
CORRESPONDENCE	reports@onehealth-analytics.org

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FROMTHEEDITOR

Why a veterinarian keeps a watch list

In companion animal practice I have examined a great many animals whose illness was, in retrospect, information. A cluster of tick-borne cases in one district. A run of bite wounds in a season when it should have been quiet. Respiratory disease in animals a few weeks before the same picture appeared in the people who lived with them. Clinical practice generates this kind of signal constantly, and almost all of it is discarded, because no one is collecting it.

The surveillance systems that do exist are built for human health departments and read human-health sources. They are excellent at what they do. But they tend to notice an outbreak at the point where people are already presenting to hospitals, which is to say after the window in which a veterinary observation might have been useful. The animal-side signal — the herd, the vector, the reservoir, the working dog, the household cat — is the part that arrives first and is watched least.

This publication is an attempt to watch it. The system behind these reports screens open scientific, governmental and news sources continuously and surfaces what appears at the animal–human interface: spillover events, reservoir dynamics, antimicrobial resistance moving through livestock, vector range shifts under changing climate. It is not a substitute for institutional surveillance and does not pretend to be. It is a second set of eyes trained on the part of the picture that a clinician notices and a database usually does not.

The period collected here made the case better than I could have. A Bundibugyo Ebola outbreak went from roughly fourteen hundred cases to more than two thousand in a fortnight and became, by the World Health Organization's own description, the fastest-growing Ebola outbreak on record. A second filovirus appeared in a toddler in Uganda while that response was already stretched. An H5N1 lineage derived from cattle was shown to infect lactating swine efficiently through the mammary gland. Two mpox clades were found co-circulating in a single province already under pressure from Ebola. None of these are human-health stories with an animal footnote. They are animal stories that became human ones.

What follows is the record of forty-five consecutive days of that watching, organised by the threads that ran through it.

Jae Seok Bae, DVM
Editor · One Health Analytics LLC

THE RECORD

Forty-five days, no gaps

The publication record below is drawn from the sending records of the distribution platform and can be verified against the public archive at archive.onehealth-analytics.org.

SERIES	ISSUES	FIRST	LATEST	DESIGNATION
Daily Surveillance Brief	45	2026-06-16	2026-07-30	[OHA Daily] YYYY-MM-DD
Weekly Surveillance Review	6	2026-06-22	2026-07-27	[OHA Weekly] YYYY-Wnn
Monthly Review	1	2026-07-01	2026-07-01	[OHA Monthly] Month YYYY
Total	52	2026-06-16	2026-07-30	

Every issue is generated and transmitted by an automated scheduler, independent of the editor's working hours or location. The daily series has been issued on every calendar day since 16 June 2026 without an omitted issue.

Collection system

GENERATION	SYSTEM	PERIOD	COLLECTION CAPACITY
First	Scout / OHZDIS	April – May 2026	27 active sources
Second	Guardian Watch Worldwide	June 2026 –	109 registered sources; 82–96 active in a given week

Both generations are the editor's original work and are the subject of registered copyrights in the Republic of Korea. Weekly issues in this period screened between roughly fifteen thousand and sixty thousand items each, drawing on between nineteen and ninety-six active sources depending on the reporting configuration in use.

Scope of collection over the period

WEEK	SIGNALS	ITEMS SCREENED	ACTIVE SOURCES
2026-W27	372	15,930	82
2026-W28	1,026	29,232	94
2026-W29	2,840	60,423	90
2026-W30	1,631	54,935	96

THE PERIOD IN REVIEW

An emergency, and everything around it

One story dominated these forty-five days, and the more interesting question is what it made visible about everything else.

The Bundibugyo ebolavirus outbreak spanning the Democratic Republic of the Congo and Uganda escalated through the period from roughly fourteen hundred confirmed cases at the start of July to more than two thousand by mid-month, and past a thousand deaths by late July. The World Health Organization described it as the fastest-growing Ebola outbreak on record. It became the third-largest Ebola outbreak ever recorded, then kept going.

> *The outbreak did not simply grow. It exposed what the response was standing on — unpaid health workers, absent diagnostics, borders that closed and reopened on political rather than epidemiological logic.*

Around that centre, the period produced a set of signals that would each merit attention on their own. Uganda detected Marburg virus in a toddler while it was still managing Ebola spillover at its own border — two high-consequence filoviruses in one region, competing for the same laboratory and contact-tracing capacity. An H5N1 lineage derived from dairy cattle was shown experimentally to infect lactating swine efficiently by the mammary route, and infectious H5N1 was isolated from commercial fetal bovine serum in the United States, raising a biosafety question about biological products that had not previously been on anyone's list. Genomic surveillance found mpox Clade Ia and Clade Ib co-circulating in South Kivu, a province already absorbing the Ebola response.

Further out, the picture was of range expansion rather than acute crisis. Crimean-Congo haemorrhagic fever surged in Afghanistan, intensified in Iraq, and was characterised in ticks from Cameroon and in Central Kazakhstan — a disease consolidating across an arc from Central Asia into sub-Saharan Africa. Australia recorded its first confirmed H5N1 detection in a wild bird. Usutu virus turned up in southern Italy. Rabies was reported in Cape fur seals, described as an unprecedented shift in the ecology of a disease we thought we understood.

And a hantavirus cluster appeared aboard a cruise ship. Andes virus is one of the few hantavirus strains with documented limited person-to-person transmission; encountering it in a travel context rather than a rodent-exposure context is precisely the kind of anomaly that a keyword screen catches and a specialist reading their own literature might not.

The sections that follow take each of these threads in turn, in the order the reporting gave them weight.

BUNDIBUGYOEBOLAVIRUS

The defining emergency

Bundibugyo is one of the less commonly seen Ebola species, distinct from the Zaire species responsible for the large West African epidemics of the past decade. By the end of this period it had produced the fastest-growing Ebola outbreak on record.

WEEK	REPORTED POSITION
W27 3 Jul	Approximately 1,406 confirmed cases and 438 deaths as of 2 July. Spread to at least four DRC provinces. Third-largest Ebola outbreak on record. France reports its first confirmed case, in a doctor returning from a mission in DRC.
W28 10 Jul	Cases surpass 1,700; deaths exceed 600. Ituri province accounts for roughly nine in ten current cases. Uganda reports no new infections for two consecutive weeks. Health workers strike over unpaid wages. WHO adds the first Emergency Use Listed diagnostic specific to Bundibugyo virus.
W29 17 Jul	More than 2,000 confirmed cases across five provinces; more than 750 deaths. WHO describes it as the fastest-growing Ebola outbreak on record. A full-scale health worker strike is threatened. A trial of both an Ebola and a Marburg vaccine begins in Uganda.
W30 24 Jul	Deaths surpass 1,000. WHO notes early and tentative signs of stabilization even as spread continues. A new variant of the Bundibugyo strain is described in correspondence in The Lancet.

What the numbers did not say

The epidemiological curve is the least interesting part of this record. What the reporting tracked alongside it was the condition of the response itself.

Health workers in DRC went on strike over unpaid wages, and the threat of a full-scale stoppage recurred through the period. Contact tracing and case management depend on those workers; a labour dispute is an epidemiological variable. A second treatment centre opened in North Kivu. Dedicated nurseries were established for children separated from infected parents — an operational detail that says more about the scale of the event than any case count.

Diagnostics were absent at the start. Bundibugyo virus had no rapid point-of-care test; WHO's Emergency Use Listing of the first such assay arrived in early July, well into an outbreak already numbering in the thousands.

Exportation stops being theoretical

On 2 July, France reported a confirmed case in a doctor returning from a DRC mission. Modelling work posted to medRxiv was already examining importation and spread dynamics into Uganda using population displacement data, work directly relevant to ring vaccination and contact-tracing priorities. By late July, Africa CDC was calling on the United States to lift Ebola-related entry restrictions on Uganda as counterproductive, while Canada imposed temporary travel measures of its own — the familiar pattern in which border policy and epidemiological evidence diverge.

The research response

Publication volume on Bundibugyo virus rose sharply through the period, consistent with a declared public health emergency of international concern. Network-based modelling examined importation into Uganda using displacement tracking data. A stochastic network study assessed ring versus community vaccination strategies. Pseudovirus analysis tested how well vaccine-induced antibodies neutralise this strain. Genomic work described mutation mapping and evolutionary dynamics of the 2026 Ugandan genomes, and a new variant was reported in Lancet correspondence.

A cost-effectiveness analysis compared geographically targeted with ring vaccination campaigns. Work on financing mechanisms argued that financial innovation could deliver a Bundibugyo vaccine — a reminder that the constraint on filovirus countermeasures has rarely been purely scientific.

Commentary broadened as the period went on. The Lancet and Lancet Microbe began framing the outbreak in terms of ecological disruption and structural vulnerability rather than pathogen control alone. A Lancet Infectious Diseases comment addressed the disproportionate risk to frontline health workers, twice. Papers appeared on the exclusion of pregnant and breastfeeding women from research, on whether community engagement was substantive or performative, and on early vulnerabilities in infection prevention and control during the outbreak's first weeks.

The economic and informational dimensions

The United Nations warned that worst-case economic costs for Africa could reach 3.6 billion dollars, and later that the toll might push close to one million people into poverty. WHO's Africa Regional Office published an infodemic monitoring report — misinformation management now a routine line item in outbreak response. A study of YouTube reactions to the emergency documented mistrust, misinformation and geopolitical anxiety, its title borrowing the phrase that surfaces in every spillover event: blame the bats again.

> *Preparedness analyses looked outward from DRC — to Somalia, to Kenya, to West Africa — asking what the outbreak revealed about readiness elsewhere.*

Papers assessed Ebola preparedness in Somalia drawing lessons from the eastern DRC outbreak, modelled waning immunity and re-infection dynamics in Kenya, and examined what the outbreak revealed about epidemic preparedness in West Africa. Institutional governance and capacity at the West Nile Uganda–DRC land border crossings was analysed directly, identifying a persistent gap in prospective standardised surveillance at high-risk crossings.

Uganda, for its part, reached a milestone: a countdown to declaring the end of its own linked Bundibugyo cases, and the start of a clinical trial evaluating both an Ebola and a Marburg vaccine — a rare piece of encouraging infrastructure news in a period that produced little of it.

MARBURG

The second filovirus

In early July, Uganda detected Marburg virus in at least one case — a toddler. The country was at that point still managing cross-border pressure from the Bundibugyo outbreak.

Marburg is a filovirus related to Ebola, and its appearance in Uganda under those conditions created a response environment with few precedents: two high-consequence filovirus events in the same region, drawing on the same laboratory capacity, the same contact-tracing personnel, the same stretched budgets. Africa CDC simultaneously called for approximately eighteen million dollars in additional funding to support critical research responses, a figure that reflects the strain on continental outbreak infrastructure rather than the cost of any single intervention.

The situation appeared to be at a very early stage and its full scope was not clear within the period. What the reporting could establish was the significance of the co-occurrence.

Regional memory as a resource

A Lancet Global Health health-policy piece drew on Rwanda's recent Marburg response to offer practical lessons for strengthening Africa's capacity to manage viral haemorrhagic fever outbreaks more broadly — timely given the concurrent pressures. Separate work examined Rwanda's National Reference Laboratory operational capacity during the 2024 Marburg response.

Retrospective material accumulated through the period: a descriptive study of the epidemiology and containment of Ethiopia's first Marburg outbreak in 2025, molecular and epidemiological analysis of that same event framed explicitly around preparedness gaps and historical lethality, and a case report from the Adansi North district of Ghana's Ashanti Region. WHO published interim guidance on diagnostic testing for both Ebola and Marburg virus disease on 9 July.

Bacterial coinfection

One thread ran across the filovirus reporting and is easy to overlook. A review addressed bacterial infection in Ebola virus disease, Marburg virus disease, Crimean-Congo haemorrhagic fever and Lassa fever together. Secondary bacterial infection in viral haemorrhagic fever is a clinical management question that receives a fraction of the attention given to the viruses themselves, and it appeared repeatedly in the daily reporting during the last week of July.

INFLUENZA A (H5N1)

The host range keeps widening

No human cases drove the H5N1 signal in this period. What drove it was geography, and the animals in which the virus was turning up.

New ground

Australia notified its first confirmed detection of highly pathogenic avian influenza H5N1 in a wild bird through WOAH — a country with historically limited exposure to the virus in wild bird populations. India reported H5N1 in poultry in the Alappuzha and Kozhikode districts of Kerala, triggering culling operations. Israel reported detection in a turkey flock.

Genomic surveillance documented silent spread of clade 2.3.4.4b among wild birds and mammals along Brazil's southern coast, and a pan-continental spillover risk analysis covered Africa. A separate analysis traced the rapid expansion of genotype D1.1 across North American flyway regions, and another documented the virus in South America between 2022 and 2025, including southward expansion into the Antarctic region.

A twenty-year analysis published in Frontiers in Cellular and Infection Microbiology reinforced that the virus continues to link the Caspian Basin, Eurasia and Africa in ways that complicate containment.

The mammary route

Bovine-derived H5N1 efficiently infects lactating swine via the mammary gland.

That finding, posted to bioRxiv in late July, was the highest-scoring signal of its week. The bovine H5N1 episode had already established that the mammary gland is a productive site of replication in cattle. Demonstrating that the same lineage can infect lactating swine efficiently by the same route raises the prospect of an expanding mammalian host range and a new amplification pathway — swine being the classical reassortment vessel. Separate work reported detection of clade 2.3.4.4b genotype D1.2 in swine after experimental inoculation.

Against this, a study of evolutionary constraints limiting additional mammalian adaptation of bovine-derived H5N1 in ferrets offered a partial counterweight, suggesting the path to further adaptation is not unobstructed.

A biosafety question

A report in CDC's Emerging Infectious Diseases described the isolation of infectious H5N1 virus from commercial fetal bovine serum in the United States in 2025. Fetal bovine serum is a ubiquitous laboratory reagent. The finding raises questions about laboratory biosafety and the integrity of biological products that sit outside the usual frame of avian influenza surveillance entirely, and it is the kind of item that a veterinary-facing screen surfaces because it sits at the junction of animal agriculture and laboratory practice.

MPOX

Two clades, one province

A preprint posted in the second week of July reported the co-circulation of both Clade Ia and the more recently identified Clade Ib in South Kivu, DRC, from January through May 2026.

South Kivu was at the same time reporting more than 3,900 mpox cases for the year and absorbing a share of the Ebola response. The simultaneous presence of two mpox clades in an area under that degree of strain adds diagnostic and response complexity that neither clade would present alone.

Nationwide genomic surveillance work described multiple Clade Ib introductions together with APOBEC3-driven evolution and terminal deletions. The APOBEC3 mutational signature is the marker of sustained human-to-human transmission rather than repeated zoonotic introduction — which makes it, in surveillance terms, the finding that changes what the case counts mean.

On the countermeasure side, a phase-3 trial protocol was published evaluating the MVA-BN vaccine in very young children, aged four months to under two years, in DRC. Additional trials addressed dose-reduction strategies and immunogenicity in children from two to under twelve years. A simplified antigen-based serological algorithm for classifying exposure and vaccination status also appeared.

One item sat at the animal interface directly: detection of anti-orthopoxvirus neutralising antibodies in three dogs and one cat from a household with a confirmed human mpox case in Brazil.

NIPAH

From detection to anticipation

Two substantial pieces appeared in the same week in mid-July: a paper in PLOS Neglected Tropical Diseases and a systematic review in Lancet Infectious Diseases. The former argued for a fundamental reframing of early warning in Bangladesh — away from reactive detection and toward anticipatory surveillance.

Late in the period, a systematic review inferred transmission parameters across Bangladesh and India from 2001 to 2026: reproduction number, offspring dispersion, and serial interval. Offspring dispersion is the parameter that governs superspreading potential, and the review's implication was continued endemic risk with capacity for superspreading events in South Asia.

Supporting work through the period examined Pteropus medius — the reservoir bat — from a One Health perspective covering spillover, microbiota and antimicrobial resistance together; assessed knowledge gaps in surveillance, prevention and health system response; and produced an international risk assessment of Nipah outbreak with medium-to-long-term prevention prospects in China.

HANTAVIRUS

A rodent-borne virus goes to sea

A WHO Disease Outbreak Notice flagged a hantavirus cluster linked to cruise ship travel across multiple locations. It was the highest-scoring signal of its week.

Hantavirus is typically transmitted through rodent exposure, not person to person. Its appearance in a travel context is the anomaly. Andes virus is among the rare hantavirus strains with documented limited human-to-human transmission, which distinguishes it from most others and makes a shipboard cluster a materially different proposition from a rural exposure event.

The literature responded quickly. A paper on differentiated contact tracing for emerging zoonotic hantavirus and ebolavirus infections drew lessons from both the 2026 Andes virus event aboard the MV Hondius and the Bundibugyo outbreak. A multilayer network study modelled propagation speed in a cruise-ship case. Governance and operational lessons from the MV Hondius event were examined under the heading of expedition cruises, island hopping and zoonotic risk — an item that recurred across several weeks. A discrete choice experiment probed public preferences for pandemic response strategies in a hypothetical hantavirus outbreak.

In the same week, WHO declared an earlier hantavirus outbreak over, commending Spain for its response. Later in the period Venezuela confirmed three hantavirus deaths, and a Lancet Microbe commentary asked whether hantavirus surveillance is asking the right questions. Work on ecological cascades and future hantavirus spillover risk in a changing climate rounded out the thread.

CRIMEAN-CONGOHAEMORRHAGICFEVER

An expanding arc

CCHF is tick-borne and already endemic across a wide band from Central Asia to sub-Saharan Africa. The period recorded it consolidating.

Iraq's CCHF season intensified in 2026. Afghanistan reported a 174 percent increase in cases in June 2026 against the prior period — a striking rise in a setting with limited surveillance infrastructure. Integrated tick and animal surveillance characterised the virus in Central Kazakhstan, adding detail to its footprint in the region. Genomic work characterised CCHF virus in Hyalomma ticks from Cameroon, highlighting circulation of the Africa 3 lineage. And a global portrait of the virus in wild ungulates documented host range expansion with direct implications for zoonotic transmission.

Taken together these are not four separate national stories. They are one distribution widening, observed from four angles.

Foodborne, vector-borne, and the long tail

Foodborne

A cyclosporiasis outbreak in the United States grew to more than a thousand cases and prompted a CDC Health Alert Network notice; by late July a multistate Cyclospora cluster had been linked to lettuce served at Taco Bell locations, a recurring summer-produce pattern. Hepatitis E virus RNA was detected in processed pork products in Spain in a year-long prevalence study. A Salmonella enterica serovar Braenderup outbreak was reported from Guangdong, and a serovar Reading outbreak in New South Wales was linked to dried bovine meat. Hong Kong's Centre for Health Protection investigated a case of human infection with rat hepatitis E virus.

Vector-borne and arboviral

Chile's health ministry began standing up sentinel surveillance sites for arboviruses, reflecting concern in southern South America where such diseases have historically been uncommon. A CDC travel notice elevated chikungunya in Costa Rica to a Level 2 alert; vector competence data from Brazil showed local Aedes albopictus transmitting both East-Central-South African and Asian genotypes efficiently. The Philippines recorded hundreds of suspected chikungunya cases in Samar province. Usutu virus was detected in southern Italy with phylogenetic work on introduction and spread routes. Oropouche virus continued circulating in the Colombian Amazon. Mexico reported leishmaniasis cases up roughly 296 percent against the same period the previous year, a figure the source itself flagged for cautious interpretation given possible changes in reporting practice. Western equine encephalitis virus reemerged in Argentina, including detection in blood donors.

Antimicrobial resistance

AMR signals recurred throughout, spanning livestock manure, slaughter cattle, broiler production chains and piggery effluents, and covering Salmonella, Streptococcus suis, Campylobacter coli and Yersinia pestis. The consistent editorial reading was that these require integrated genomic surveillance linking animal, food and human isolates rather than parallel monitoring streams.

The unexpected

Rabies was detected in Cape fur seals, described by WOAH as an unprecedented shift in rabies ecology. Nairobi sheep disease virus — a tick-borne pathogen not commonly associated with humans — was reported in the upper respiratory tract of people in Beijing, preliminary but unusual enough to note. Trypanosoma cruzi was identified in bats from Belize using metagenomic sequencing, with neotropical bats framed as sentinels for emerging zoonoses. An anthrax outbreak in Uganda's Kanungu District, linked to consumption and handling of livestock carcasses that had died suddenly, served as a reminder that classic zoonotic hazards remain active while attention is elsewhere. And Anaplasma capra was detected in goats in Egypt for the first time.

WEEKBYWEEK

Lead signal of each weekly review

WEEK	TOP-RANKED SIGNAL	SCORE
W27	Hantavirus outbreak linked to cruise ship travel, multiple locations · WHO Disease Outbreak News	33
W28	Population mobility and Ebola case containment in Ituri, DR Congo · The Lancet	25
W29	Ebola Bundibugyo virus disease outbreak, DRC and Uganda — Weekly External Situation Report 08 · WHO AFRO	30
W30	Ebola disease caused by Bundibugyo virus, DRC and Uganda · WHO Disease Outbreak News	27
W31	Transmission dynamics of Nipah virus in Bangladesh and India, 2001–2026 · medRxiv	29

Watch list carried into the next period

THREAD	WHAT TO FOLLOW
Bundibugyo Ebola	Whether WHO's early stabilization signals are confirmed by case counts; whether the health worker dispute is resolved; ring vaccination rollout data; how travel restrictions evolve.
H5N1, bovine lineage	Further experimental or field evidence of mammary-route transmission in swine and other mammals; reassortment risk; regulatory response to the fetal bovine serum finding.
Mpox Clade Ib	New country introductions; continued viral evolution as read through APOBEC3 signatures and deletion events.
Nipah	Whether the updated reproduction number and dispersion estimates prompt revised outbreak response protocols ahead of the next seasonal spillover window.
CCHF	The Afghanistan surge, given its scale and the limits of local surveillance infrastructure.
Hantavirus	Any further cruise-ship-associated cases, which would substantially change the risk profile of that signal; the Venezuela cluster.

PUBLICATIONLOG

Complete record, 16 June – 30 July 2026

All fifty-two issues, in order of publication. Every issue is readable in full at archive.onehealth-analytics.org.

NO.	DATE	DESIGNATION	NO.	DATE	DESIGNATION
1	2026-06-16	[OHA Daily] 2026-06-16	27	2026-07-08	[OHA Daily] 2026-07-08
2	2026-06-17	[OHA Daily] 2026-06-17	28	2026-07-09	[OHA Daily] 2026-07-09
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26	2026-07-07	[OHA Daily] 2026-07-07	52	2026-07-30	[OHA Daily] 2026-07-30

INCLOSING

The next forty-five days

Looking back over this period, one thing repeats. Whatever the story, it was visible on the animal side before it reached people.

Ebola and Marburg have bats as their reservoir. H5N1 moved from wild birds to poultry, to dairy cattle, and now to the mammary gland of swine. Nipah begins in fruit bats, hantavirus in rodents, Crimean-Congo haemorrhagic fever in ticks. Rabies in Cape fur seals, Trypanosoma cruzi in bats from Belize, Anaplasma capra in Egyptian goats are not human stories yet. Some of them will be, and by then it will be late.

What this publication tries to do is keep a record of that earlier stretch. Not a diagnosis and not an alarm — a record. If what appeared, and when, and where, is written down, it becomes usable material for whoever later has to decide what it was.

Reading the figures

Saying how active a signal is relative to normal requires that signal's own history. We are building that baseline now, which is why the reports in this period presented baseline comparisons cautiously and, where the evidence was not yet there, declined to present them at all. Not asserting ahead of the data is the first discipline of surveillance work. The comparison sharpens as observation accumulates.

Collection also widened quickly, from nineteen active sources in one week to ninety-six in another. Weekly signal counts therefore reflect both disease activity and the growth of observing capacity. With that in mind, this volume is organised around how each thread developed rather than around movements in the totals.

Continuing

The reports continue. The daily brief goes out every morning, the weekly and monthly editions on their fixed schedules, and they will keep going out — whatever the period brings, without a missed day. The next compiled volume, covering the autumn quarter, is due on 31 October 2026.

If this record has been useful, please subscribe, and pass it to a colleague. If you find something wrong in it, tell me and it will be corrected. Surveillance is not solitary work, and neither is this publication.

Jae Seok Bae, DVM
Editor · One Health Analytics LLC

COLOPHON

The One Health Signal is compiled quarterly from the daily, weekly and monthly surveillance reports issued by One Health Analytics LLC. Reports are generated by an automated collection system designed and operated by the editor, and are distributed on a fixed schedule independent of the editor's working hours or location.

Subscribers receive the weekly and monthly editions. The complete publication record, including every issue of the daily series, is set out in the publication log above and is publicly readable in the archive.

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This volume is a self-published compilation. It carries no ISBN or ISSN and is not offered for sale. It is distributed to subscribers as a record of the reporting issued during the period stated on the cover.

Signals are drawn from open scientific, governmental and news sources. Their appearance here indicates detection by the collection system and summarisation of the reporting source, not independent verification by the publisher. Signal scores are relative indicators of priority, not clinical risk assessments. Nothing in this volume constitutes clinical or public health guidance.

Where preprint sources are cited, those findings have not completed peer review and should be interpreted accordingly.

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